

UC Treatment Comparison Guide

This expanded comparison guide summarizes major ulcerative colitis treatment categories in plain English to help patients prepare for informed conversations with their healthcare teams.

5-ASA Medications

- Mesalamine therapies are commonly used for mild to moderate ulcerative colitis.
 - Oral and rectal formulations may be combined for left-sided disease.
 - Consistent daily use may reduce relapse risk.
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Corticosteroids

- Prednisone and budesonide are commonly used for active flares.
 - Steroids are effective short-term tools but are not intended for long-term maintenance.
 - Repeated steroid dependence may indicate the need for stronger therapy.
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Biologic Therapies

- Anti-TNF medications target inflammatory proteins involved in ulcerative colitis.
 - Vedolizumab acts more selectively in the gastrointestinal tract.
 - Ustekinumab may help patients who lost response to previous biologics.
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JAK Inhibitors

- Tofacitinib and upadacitinib are oral medications that may work faster than some biologics.
 - Monitoring for infection risk, cholesterol changes, and cardiovascular risk is important.
 - Treatment decisions should be individualized based on patient history.
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Treatment Planning Questions

- How will we measure remission and treatment success?
 - What monitoring tests are required during treatment?
 - What side effects deserve urgent attention?
 - What happens if my current medication stops working?
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Quick Reference Summary

Treatment Type	Main Consideration
5-ASA	Often first-line for mild to moderate disease
Steroids	Short-term flare control only
Biologics	Used for moderate to severe disease
JAK Inhibitors	Fast-acting oral options requiring careful monitoring

Medical Disclaimer: This educational resource is not medical advice. Always consult a licensed gastroenterologist before starting, stopping, or changing treatment.